Encompass Health Rehabilitation Hospital of Scottsdale
9630 E. Shea Blvd. Scottsdale, AZ 85260-

Patient: BENNETT, SHUMEI
MRN: 875005504 Admit Date: 3/19/2026
FIN#: 501415496 Discharge Date:
DOB/Age/Gender: 2/1/1939 87 years Female Attending Phys: Bal MD,Harjinder Singh

Face Sheet

Encompass Health Rehabilitation Hospital of Scottsdale (480) 551-5400

Encounter Information

FIN: 501415496
Admit Dt/Tm: 03/19/2026 12:32
Disch Dt/Tm:
Attend Physician: Bal MD, Harjinder Singh
Admit Physician: Sumayo DO, Royce
PCP: Turner M.D., Jason Hirst
Referring Physician: Abdulrasool SR., Mohammad Alaa
Onset of Symptoms/Illness: (11) 03/16/2026
Other Accident: (05) 03/16/2026

Third Party MRN:
Patient Type: Inpatient
Medical Service: OTH Other Conditions
Subspecialty:
Location: NS SDL 1
Room/Bed: 101 / A
Admit Reason: NEUROLOGICAL COND
Accommodation: General Private
VIP Indicator:

Patient Information

Patient Name: BENNETT, SHUMEI
MRN: 875005504
Home Address: 14413 E GERONIMO RD
SCOTTSDALE, AZ 852593784
County: MARICOPA
Home Phone: (719)231-6405

Admin Sex: Female
Birth Sex:
DOB: 02/01/1939
Age: 87 Years
Religion: Unknown
Race: Other Asian
Email Address: shumei1234@outlook.com
Mobile Phone:

Guarantor Information

Guarantor Name: BENNETT, SHUMEI
Billing Address: 14413 E GERONIMO RD
SCOTTSDALE, AZ 852593784

Patient's Reltn: SELF
Home Phone: (719)231-6405
Mobile Phone:

Contact Information

Emergency Contact
Contact Name: LAAHS, SHARON
Patient's Reltn:
Home Phone: (719)231-6405
Mobile Phone:

Emergency Contact
Contact Name:
Patient's Reltn:
Home Phone: shumei1234@outlook.com
Mobile Phone:

Primary Insurance

Insurance Name: MCR Medicare
Financial Class: Medicare
Policy Nbr/Group Nbr: 9DM7DX4WH64 /
MBI:

Claim Address:
,
Authorization Number:

Secondary Insurance

Insurance Name:
Financial Class:
Policy Nbr/Group Nbr: /
MBI:

Claim Address:
,
Authorization Number:

Tertiary Insurance

Insurance Name:
Financial Class:
Policy Nbr/Group Nbr: /
MBI:

Claim Address:
,
Authorization Number:

Comments:
FS verified. JM

Printed by: Melotte Admissions Liaison, Julie on 03/20/2026 16:02 to pr
Registration last updated by: Melotte Admissions Liaison, Julie on 03/20/2026 16:02

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History and Physical

DOCUMENT NAME: History and Physical
SERVICE DATE/TIME: 3/20/2026 16:40 MST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Bal MD,Harjinder Singh (3/20/2026 16:45 MST)
SIGN INFORMATION: Bal MD,Harjinder Singh (3/20/2026 16:45 MST)

Dr Bal H&P

Patient: BENNETT, SHUMEI MRN: 875005504 FIN: 501415496
Age: 87 years Sex: Female DOB: 2/1/1939
Associated Diagnoses: None
Author: Bal MD, Harjinder Singh

Admission Date/Time: 03/19/26 12:32

Chief Complaint: Transferred from Honor Health Shea Medical Center for improvement in the functional status

History of Present Illness: this is a very pleasant 87 year female who has known history of Parkinson disease, hypertension, hyperthyroidism. Patient lives alone and recently sustained a ground level fall leading to hospitalization where the workup found to have right periorbital ecchymosis/hematoma, nontraumatic rhabdomyolysis, non ST-elevation MI secondary to fall and rhabdomyolysis. Patient had extensive workup in the hospital including seen by Cardiology and Neurology. Patient was subsequently seen by PT and OT they recommended acute rehab. Patient got transferred to encompass Health on March 19. Patient seen in room this morning, daughter at the bedside. Per daughter patient was pretty independent until year ago when she got diagnosed with a Parkinson disease. Patient is still able to live independently.

Past Medical/ Surgical History: As outlined above

Allergies (1) Active	Reaction
No Known Medication Allergies	None Documented

Medications (3) Active
Scheduled: (3)
calcium citrate 950 mg (200mg elemental Calcium) Tab 200 mg 1 tab, Oral, Daily, Start 03/21/26 8:00:00 MST
carbidopa-levodopa 25 mg-100 mg 1 tab, Oral, TID, Start 03/19/26 14:00:00 MST
methIMAzole 5 mg Tab 5 mg 1 tab, Oral, Daily, Start 03/20/26 8:00:00 MST

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History and Physical

Continuous: (0)

PRN: (0)

Family History: not contributory

Social History: no smoking alcohol or drug abuse, lives by herself

Review of Systems: not obtainable from the patient rest as per HPI

Data review:

Labs (Last four charted values)
Cr 0.40 (MAR 19)

Diet:
Diet -- 03/19/26 22:26:00 MST, Texture: Level 7 - Regular, Liquid Consistency: Level 0 - Thin, Restrictions: No Restrictions, Special Instructions: Pills Whole

Physical Examination:

Vital Signs (last 24 hrs)	Last Charted
Temp Oral	98.8 DegF (MAR 20 06:27)
Heart Rate Peripheral	78 bpm (MAR 20 09:45)
Resp Rate	16 br/min (MAR 20 06:27)
SBP	H 165mmHg (MAR 20 09:45)
DBP	77 mmHg (MAR 20 09:45)
SpO2	98 % (MAR 20 09:45)
BMI	20.93 (MAR 19 19:15)

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History and Physical

Appearance/ Constitutional: awake and alert
HEENT: right periorbital ecchymosis present head is atraumatic
Neck: supple
Lungs: coarse bilateral
Cardiac: sinus rhythm
Abdomen: soft, nontender nondistended
Genitourinary: deferred
Musculoskeletal: no joint pain
Extremities: no edema
Skin: no rash except for ecchymosis around the eye
Neurologic: global weakness, no focal motor deficit
Cognition: appropriate for her age
Cranial nerves:

Impression and Plan:
Mechanical ground level fall
Nontraumatic rhabdomyolysis
Parkinson disease with a fluctuating manifestation
Hyperthyroidism
Essential hypertension with a orthostatic hypotension
Right periorbital ecchymosis/mild hematoma without fracture
Plan:
Inpatient hospitalization at encompass Health
PM&R consult from Dr. Sumayo noted and appreciated
Continue regular home medication
Pain control
Bowel care
Sleep aid if needed
Will follow patient closely and make further recommendation as warranted
Discussed with the patient daughter at the bedside
Code status: DNR/DNI
Complexity: High

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History and Physical

"Given the patient's rehabilitation need and on going medical issues, this patient is best served in a comprehensive inpatient rehabilitation setting with an interdisciplinary approach with at least 3 hours of skilled therapies for at least 5 days per week. Considering all the information above, it is my best judgement that this patient requires intensive therapy in an inpatient rehabilitation hospital due to rehabilitation needs. This patient can reasonably be expected to participate in and benefit in an interdisciplinary delivery of care under the direction of a rehabilitation or equally certified physician. The goal is to return the patient to as close as possible to prior function."

Electronically Authenticated on 03/20/2026 04:45 PM

Harjinder Bal, MD